
PHASE 1 OF THE ENGAGEMENT / WORKSTREAM 1

The Diagnostic Audit

Four weeks. Evidence, not impression. A costed plan of action.

The mobilisation phase of the Haven turnaround. We establish exactly where the facility stands across safety, culture, operations, and cash, and deliver two quick wins while the audit is still running.

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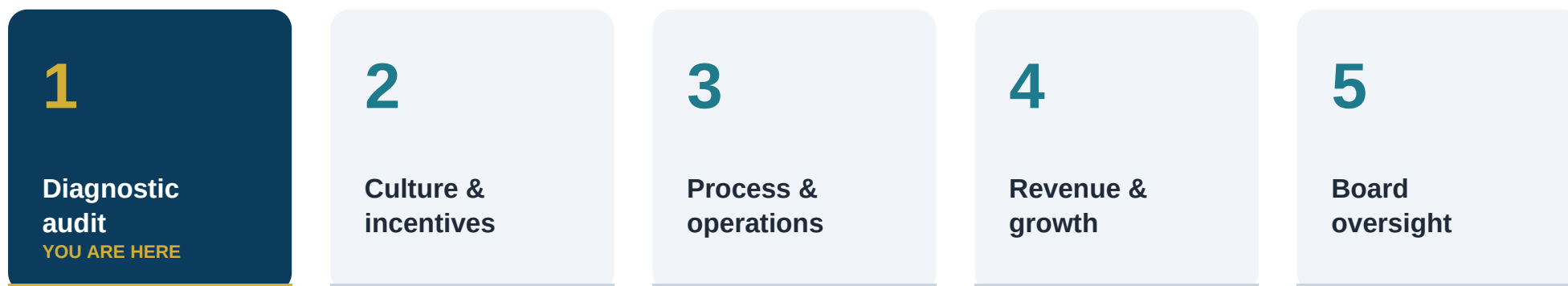
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Fee: N2,100,000 on signing (standard N3,500,000, at the 40% partner discount)



One commitment now: the audit that shapes everything after it

The engagement is one board-led turnaround with five workstreams. The board only needs to commission Workstream 1 today. Its findings shape the detail, and the price, of everything that follows.



Phase 1 is the foundation. The remaining workstreams are scoped and priced off the audit's findings.



Leave the board with proof, and a prioritised, costed plan

Establish, from evidence rather than impression, exactly where Haven stands across clinical safety, culture, operations, and cash.

01 Evidence

Findings grounded in records, observation, and interviews, not in impression or blame.

02 A costed plan

Prioritised recommendations, each costed, so the board can decide what to do and in what order.

03 Value during

Two safety and cash quick wins are delivered while the audit runs, not after it reports.



Collaborative, not audit-by-ambush

- **Structured interviews.** Leadership and frontline staff, nursing, pharmacy, customer service, and operations, so the picture reflects how work actually happens.
- **Direct observation on site.** We watch the real operating day, not just the documented one.
- **Records review.** Financial, clinical, and operational records, read against each other for what reconciles and what does not.
- **A short data request in week one.** A focused list issued on day one so analysis starts immediately, not in week three.

The team is a partner in the findings, not a target of them. Honest data depends on people feeling safe to show us the real picture.



Eight areas, one connected picture

1

**Clinical governance
& patient safety**

2

**Culture, routines
& incentives**

3

**Operations &
SOP adherence**

4

**Finance &
working capital**

5

**Procurement
& inventory**

6

**HMO
economics**

7

**People &
structure**

8

**Management
reporting integrity**

The next pages go deeper on each. The areas are examined together, because the failures connect.



Clinical governance & patient safety

The question we answer: What makes acute paediatric care dependable on every shift, and ready to scale?

- **Incident and near-miss systems.** The reporting and learning process that lets the team surface and fix risks early, the marker of mature clinical governance.
- **Emergency readiness.** Crash-cart and emergency-readiness standards, and the shift-level checks that keep them dependable.
- **Clinical protocols.** Paediatric and NICU protocols, and real adherence against them.
- **NICU readiness.** What it takes to scale admissions safely, the engagement's highest-yield growth lever.

Strong clinical governance is what lets a growing facility take on more complex care, including NICU, with the team and families confident in the standard. It is the foundation everything else builds on.



Culture, routines & incentives

The question we answer: Do the culture and incentives produce safe, efficient care without anyone chasing it?

- **How work actually gets done.** The real operating day versus what the documents say.
- **Where routines break down.** The nursing and operational routines that should run every shift, and where they quietly fail.
- **Whether incentives reward the right things.** The staff commission structure and the JDS and KPIs: do they reward quality and ownership, or only activity?
- **Accountability norms.** Who owns what across shifts, and how that is held.

The commission structure and JDS/KPIs already on the board's decision list are the levers. The audit calibrates them so the culture sustains itself.



Operations & SOP adherence

The question we answer: Where does the operating model leak time, money, and goodwill?

- **Patient flow, end to end.** From arrival through care to discharge and billing.
- **Customer service and booking.** The experience that drives repeat visits and referral.
- **SOP adherence.** The gap between what is documented and what actually happens.
- **Bottlenecks.** The points that cost time, margin, or patient experience.

Thin margins on reasonable revenue point to leakage in the operating model, not to a pricing problem. We find where it leaks.



Finance, working capital & HMO economics

The question we answer: Where is the cash, and how fast can we free the ~N7M that is locked up?

- **Revenue mix and yield.** Private versus HMO, and the true margin by service line.
- **The real pharmacy margin.** Separating genuine margin from the 201% figure that is a markup, not a margin.
- **~N4.2M in receivables.** Leadway and NEM ageing and recoverability, close to a full period's revenue.
- **~N2.77M in pharmacy stock.** How much is working capital that can be released.
- **HMO contract economics.** Terms, claims, ageing, and yield per HMO versus private.

The cash problem is a working-capital problem, not a profit problem. The money is on the shelf and in the HMO ledgers, and it is recoverable.



Procurement & inventory

The question we answer: Can inventory keep every critical item available and lift margin at once?

- **Stock management.** Reorder discipline and the controls that keep critical items reliably in stock.
- **Procurement reliability.** Supplier terms, lead times, and single points of failure.
- **Margin leakage.** Where pharmacy margin is lost between purchase and sale.
- **The case for vendor-managed inventory.** Whether a VMI model, such as Medbury Pharma, keeps stock dependable and lifts margin together.

Reliable inventory does two jobs at once: critical items are always available, and pharmacy margin lifts. A vendor-managed model delivers both.



People, structure & reporting integrity

The question we answer: Is the team structured to run safely, and can the board trust its own numbers?

- **Staffing and roles.** Ratios, role clarity, and where structure creates risk or cost.
- **The senior operations leader spec.** The role defined here, so it can be recruited into clarity afterward, sourced through CadreHealth.
- **Reporting integrity.** Whether the numbers the board sees can be trusted: visit counts that do not reconcile, and a receivables table that does not foot.
- **Single source of truth.** What it takes to give the board a report it can act on.

You cannot manage what you cannot measure reliably. Fixing the reporting layer is the precondition for every other decision the board makes.



Two quick wins, delivered during the audit

The board sees safety and cash improve in week one, before any findings are written up.

1. A crash cart you can trust

Stocked, sealed, and checklist-governed, with a shift-level check routine, live in week one. A core safety standard put in place from the start, not deferred to the end.

2. Cash starts moving

An immediate recovery push on the largest HMO receivables, Leadway and NEM, so money starts coming in before the audit even reports. The work begins to pay for itself.



Two deliverables, presented to the board

Both are walked through with the board in a working session at the end of week four, not emailed as a document to file away.

1. Operational & Clinical Governance Audit Report

Findings across every area examined, each with a prioritised, costed recommendation, so the board can act in order of impact and decide the shape of the workstreams that follow.

2. Working Capital & Receivables Recovery Plan

A concrete plan to unlock the cash tied up in the ~N4.2M of receivables and ~N2.77M of pharmacy stock, with the routines that keep it from locking up again.



Mobilise in week one, present in week four

Week 1

Mobilise

Stakeholder interviews, data request issued, crash-cart standard live, receivables push begins

Week 2

Care & operations

Clinical governance and safety, culture and routines, operations and SOP walkthrough

Week 3

Money & numbers

Finance and working capital, procurement and inventory, HMO economics, reporting integrity

Week 4

Synthesis

Prioritised, costed recommendations, and the board presentation



A light lift on your side, set up in week one

- **A named point person.** We suggest the Head of Operations, as the single channel for the engagement.
- **Read access to the records.** Revenue, receivables ageing and P&L, pharmacy stock data, HMO contracts, the staff roster and job descriptions, clinical incident records, and existing SOPs.
- **Time with the people who do the work.** Leadership, plus the nursing, pharmacy, and customer-service leads.
- **Walk-through access to the facility.** So observation reflects the real operating day.

NEXT STEP

Commission the audit. We mobilise within the week.

The diagnostic audit is N2,100,000, payable on signing (standard rate N3,500,000, at the 40% partner discount). On the board's go-ahead, we issue the data request, set the crash-cart standard live, and begin the receivables push, all in week one.

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